UC-01: Request / Book Appointment	
Primary Actor	Patient or Administrative Staff
Stakeholders and Interests	**Patient**: Wants to find an available spot and book an appointment with the required professional
	Medical Professional: Wants to see their schedule with the right patient, booked appointment being in their available times
	Insurance: Requires accurate patient and appointment verification records in order to do any kind of coverage
Preconditions	- The actor is authenticated and authorized to request an appointment. - The patient can be uniquely identified in the system
Success Guarantee (Postconditions)	A new 'Appointment' instance is created with a unique appointmentID, is associated with the patient and medical professional, has its status set to confirmed, and is successfully recorded in the system
Main success scenario (or basic flow)	1. The actor starts an appointment request specifying the patient ID, professional ID, preferred time, and reason (requestAppointment). 2. The system verifies the healthcare professional's availability and applicable scheduling constraints. 3. The system verifies if a medical referral is required depending on the service type. 4. The system retrieves and presents available appointment options matching the request. 5. The actor selects an available appointment time slot (selectSlot). 6. The system checks that the selected slot is still available. 7. The actor confirms and saves the appointment (confirmAppointment). 8. The system creates the Appointment instance with a unique ID, links it to the patient and medical professional, marks its status as confirmed, and records the successful booking
Extensions (or alternative flows)	- If a medical referral is required but missing or invalid, indicate an error and abort the request until a valid referral is established. - If no suitable appointment options exist matching the request, indicate an error message and suggest alternative dates or times. - If the selected slot is no longer available, indicate an error and prompt the actor to select a new slot.
Special requirements	Command Line Interface (CLI) implementation with role-based access control.
Technology and Data Variations List	Patient lookup via health insurance number (RAMQ/private), date of birth, phone number, or partial name.
Open issues	How to manage scheduling conflicts when professionals practice across multiple clinic locations.

UC-02: Record Consultation	
Primary Actor	Medical Professional
Stakeholders and Interests	**Medical Professional**: Wants a secure way to document clinical notes, diagnoses, and treatments for accountability and the record
	Patient: Wants accurate records detailing their clinical visit and care instructions
Preconditions	- The medical professional is authenticated and authorized to record clinical information. - The target appointment exists, has an arrival registered, and is valid for consultation.
Success Guarantee (Postconditions)	A new ‘Consultation’ instance is created with a unique consultationID, associated with the correct appointment, capturing observations, diagnosis, treatments, follow-up information, and recording the professional's identity.
Main success scenario (or basic flow)	1. The medical professional selects a valid appointment on the day of the visit. 2. The medical professional initiates recording consultation details (recordConsultation). 3. The medical professional enters clinical information, including observations, diagnosis, treatments, and follow-up requirements. 4. The system validates that the provided consultation information is complete and valid. 5. The system creates a unique consultationID, records the timestamp, associates it with the appointment, saves all clinical data, and logs the professional's identity.
Extensions (or alternative flows)	If required consultation information is missing or invalid, indicate an error and prompt the medical professional to provide valid data.
Special requirements	Strict accountability logging; identity of recording professionals must be securely saved.
Technology and Data Variations List	Clinical details entered as structured text via CLI interface
Open issues	Specific text-formatting requirements for storing structured diagnoses versus free-text observations.

UC-03: Create Referral	
Primary Actor	Medical Professional
Stakeholders and Interests	**Medical Professional (Referring):** Wants to officially refer a patient to a specialist or specialized facility with clear clinical justification.
	Patient: Wants a seamless referral process with tracked priority for specialized care.
Preconditions	- The patient exists in the system - The medical professional is authenticated and authorized to create referrals
Success Guarantee (Postconditions)	A new 'Referral' instance is created with a unique referralID, associated with the patient and professional, recording the clinical reason, priority, creation date, and status as saved and available.
Main success scenario (or basic flow)	1. The medical professional initiates a referral for a patient (createReferral). 2. The medical professional provides the clinical reason, requested specialty or professional, priority level, and mandatory referral details. 3. The system validates patient existence, professional authorization, and input validity. 4. The system creates a unique referralID, links the referral, records creation date, status, clinical reason, and priority, and saves it in the system.
Extensions (or alternative flows)	If the clinical reason is missing or priority/mandatory information is invalid, indicate an error and prompt the medical professional to correct the input.
Special requirements	Must support referencing external physicians who do not offer services directly within the clinic.
Technology and Data Variations List	Priority levels and statuses managed as system-defined enumerated values
Open issues	How the system will track and link a single referral that results in multiple subsequent appointments.